

Soft Regulation™ Desensitize

Q&A; Session #1 — Student Questions & Answers

Answers by Maggie Sterling

This document summarizes the first monthly Q&A; session. Questions were submitted by students and consolidated where multiple people asked similar things. Maggie notes key themes at the end — please read through to the end.

Q1 • 13 people asked

What does allowing actually look like in practice? (Bracing vs. allowing, allowing when symptoms are 24/7, allowing thoughts vs. sensations, and what to do with your mind while allowing)

Bracing vs. Allowing

The overall goal is a softening of your reaction — moving from fear and terror toward anything closer to neutral. We want to put down any form of resistance: fixing, searching, reassurance-seeking, trying to force things to change. This is so much less about allowing the right way and more about removing all the ways you are resisting and fighting your current experience.

Allowing when symptoms are 24/7

Ask yourself whether your symptoms are actually the same exact intensity 24/7, or if they move or change at all. If they truly are constant, how much focus are you placing on that? Constant focus creates additional fear. The goal remains responding with less fear and resistance. Acceptance doesn't mean accepting this as your destiny — it means accepting your current reality so you can respond differently to it.

Allowing thoughts vs. sensations

It's all the same. The more you try to categorize this into different buckets, the longer it will take. Ask yourself: in this moment, is what I am doing signaling safety or danger? Whenever you go to fix or figure it out, the default answer is danger. The danger alarm stays on because you won't stop trying to fix it.

What to do with your mind while allowing

Very little internal monologue is recommended. Talking yourself down sends more danger signals because you're trying to calm down from something alarming. The course uses a show vs. tell method — you let your thinking update on its own by living differently, which sends a different story to your brain. Many beliefs change on their own once you start living in a new way.

Q2 • 8 people asked

How do I know if my symptoms getting worse means I'm on the right track or doing something wrong? Is there a point where worsening symptoms signal a real problem?

Everyone's experience is vastly different. Some people experience fast relief; others don't — don't compare your story. You will more than likely have flares until the very end, and those flares can be worse than initial symptoms. It is highly unlikely you are doing something wrong if you are actively applying the principles taught in this course.

When you start freaking out about recalibration, or symptoms spreading or worsening, and you make that mean something is wrong — that is you getting back in the loop of signaling danger. This process will test you repeatedly. The goal is to know what to expect so that when it pops up, you can say: I know what this is, and I know it's working.

We can never know for certain you're on the right track. But the principles of desensitizing your nervous system are universal. It is your job to apply them consistently and learn to live with a level of discomfort you won't like. There's no avoiding that in this process.

Q3 • 5 people asked

How do I apply this when my symptoms are worst first thing in the morning or at night?

It's interesting to note the wide variance here — many people have the worst symptoms in the morning, others worsen as the day progresses. Either way, the work is the same: what is your response when symptoms are loud?

You do not need to just sit there and bake in your symptoms. Living your life IS the exposure to your symptoms — they're doing whatever they're doing while you go about your day. Get up and engage with your life.

Watch for the anticipatory fear loop: dreading 3pm because evenings are always bad, or waking up with doom before you've even assessed how you feel. That conversation keeps the danger loop running. Remember: you're always working on your response to the fear, not your response to the symptoms. Symptoms can exist without the fear — and once they do, they start turning down. You will recover from the fear first.

Q4 • 6 people asked

How do I know what's avoidance versus a genuine need to rest or stop? Where's the line between pushing through and reacting?

There is no black and white answer. The overall goal is to show the nervous system there is no danger. If you are constantly retreating as soon as symptoms start, you are reinforcing that they are dangerous and using them as a signal to stop — which is its own loop.

You will need to experience things your brain believes are fully not doable. A flare when you try something does NOT mean you can't do it — it means your brain has flagged it as dangerous. That is the moment to rewire. A symptom flare is not confirmation that something can't be done.

That said, don't throw yourself into irresponsible situations. You are the CEO of this process. Be responsible about pace. But also: you cannot avoid the fact that everything can't be flagged as dangerous and expect things to change. You have to begin rebuilding trust with yourself. When symptoms flare during an activity, work on your response to that flare — not on confirming you "can't" do that thing.

Q5 • Multiple people asked

Should I stop all my other healing modalities — coaching, therapy, Curable, journaling, podcasts, recovery stories?

Maggie decided in October 2024 to go all in on going all out — no more tools, techniques, five-step processes, protocols, recovery stories, or compulsive healing behaviors. For her, healing had become a full-time compulsive job. She took four months completely off from all of it.

Please stop consuming everyone's content everywhere on the internet. Pick a path — even if it's not this one — and shut out all the other noise. Different approaches create doubt and confusion. The reassurance you're getting from this Q&A; is the right kind: it reminds you what you're doing and why things are happening, without feeding the compulsion.

Examine the energy driving the other work. If you are severely sensitized, be brutally honest with yourself. Use your desperation as a gauge: if you cannot use a tool without the energy of "I need this feeling to stop," that is still a reactive behavior. Wait until you can use tools from a place of support, not desperation. Meditation, journaling, and other tools are beautiful from support — not from desperation.

As for therapy: that is outside the scope of this course. But ask yourself — is what I'm doing with my therapist adding more pressure to an already pressured system? Would it help to discuss that with them?

Q6 • 7 people asked

Does this approach still apply if my symptoms have a real cause or diagnosis (e.g., lupus, hypoglycemia, genetic connective tissue conditions)?

Your condition is real. You can have a real diagnosis AND a sensitized nervous system — these two things very often coexist. This course is not symptom-specific and does not treat your condition.

Sensitization is a layer on top of sensations or conditions. It's what turns the volume up on everything. When you add hypervigilance, fear, body scanning, and life-shrinking around your symptoms to an existing diagnosis — that layer is the sensitization. This work targets that layer.

The reactive patterns are the same regardless of what caused your symptoms. Removing that hypervigilance layer can meaningfully improve how unmanageable your condition feels. Continue working with your medical providers on the condition itself, while working on the nervous system layer here.

Q7 • 5+ people asked

Can you walk us through what allowing actually sounds like in your head in a real moment? What is the internal monologue?

Very little internal monologue is recommended — and that was intentional. The thoughts already happening are being triggered by the sensitization. Talking yourself into allowing or talking yourself down from a symptom isn't effective at that stage because your body doesn't believe it.

If there was any thought, it sounded like: "My body knows what to do with this. What was I about to do? What's next for my day?" That's it. No meaning-making, no trying to make sense of what was happening.

Allowing is not a mental thing — it is a physical softening. The most actionable parts of allowing are the things you stop doing. Don't Google it. Don't text someone for reassurance. Don't open ChatGPT. Don't try to figure it out. Decide: I'm going to let this be here without my interference. That includes stopping the mental interference of thinking there's a right way to talk to yourself.

Show vs. Tell

The course teaches a show vs. tell method. You show your system something different by living differently. You let your thinking update on its own. The goal is to get out of the way — and that is far more about non-response than any specific action.

Q8 • 4 people asked

Is it okay to continue using medication (SSRIs, benzos) while doing this work? Does medication disqualify or complicate the approach?

Medication does not disqualify you. You can do this work alongside medication. It's very rare that someone isn't put on meds at some point during a difficult sensitization journey.

If you are considering tapering, do extensive research on a hyperbolic taper and be aware that sensitization symptoms may increase as you taper — especially if you are already sensitized. This is not a separate or uniquely different situation; it falls under the same umbrella of a nervous system that's overreacting and shooting off alarms. Handle it the same way.

Note: this is not medical advice. Decisions about medication should always be made in partnership with your medical provider.

Q9 • Multiple people asked

How do I allow when symptoms are present 24/7 — not just when they flare? Does the approach change when there's no baseline of relief?

First: gently ask yourself whether your symptoms truly never change in intensity, timing, or location — or whether that's the fear talking. Even when symptoms feel constant, there are often shifts.

That said: even if your reality right now is 24/7 constant symptoms, the work is the same. Start where you are. Acceptance means not looking at this like it should be different — it's your current reality. We don't worry about the symptoms. We worry about your response to the fear about the symptoms.

24/7 symptoms are just a different starting place, not a disqualifier. You still have the ability to work on your response to the fear, even before you can get much life-living in. You can decide tomorrow to view and respond to this differently. That changed response IS the work, no matter where you are on the scale.

Q10 • Multiple people asked

How do I allow panic attacks specifically? They feel impossible to sit with.

The course does not teach sitting with things. You are not supposed to sit there and observe it. Allowing isn't about white-knuckling through — and it applies everywhere, including panic attacks.

You don't control what comes first. A panic attack will teach you quickly how little control we have over physical responses. But what would it look like to let the panic attack pass through without the added commentary about how scary it is, and without the meaning-making about the sensations?

"Sensation without explanation" (a phrase from Russell Kennedy) applies here. The sensations of a panic attack — hyperventilation, heart palpitations, numbness — are sensations. The terror is the layer we add. Panic attacks are designed to feel terrifying; that is their entire point. But feeling terrifying does not mean they are dangerous. Look at your history: you have made it through every single one.

Q11 • Multiple people asked

Is allowing the same for thoughts as it is for physical sensations? Does this apply to anxiety, OCD, intrusive thoughts, and health anxiety?

Yes — it's all the same. Anxious thoughts, intrusive thoughts, OCD loops — these are symptoms of sensitization. They follow the same rules. Let them exist without getting involved, adding commentary, or rushing to fix.

The goal is to stop taking sensations and thoughts as a signal that you must do something about them. When something upticks, instead of rushing to find a solution: "That's none of my business. My body knows what to do with this. Moving on." Incredibly hard at first — gets much easier over time.

Q12 • Multiple people asked

How do I tell the difference between allowing and ignoring? Ignoring feels like suppressing, and allowing feels passive.

The word "ignore" is never used in this course. Ignoring = running away, distracting, pretending symptoms don't exist. Allowing = making it okay that they are there. These are very different things.

Think of it as opening up to versus contracting away from. Allowing says: what's happening in my body is okay right now. It doesn't need to be different than it is. You don't need to be in your head repeating "it's okay that this is here" — when you're severely sensitized, your body won't believe it anyway. Allowing is more in the body than in the mind. It's an energy of coexistence, not a mental exercise.

Q13 • Multiple people asked

What are the signs that I'm moving from sensitized to desensitized? And when is it okay to add other tools back in?

Signs of progress

Progress in the beginning won't look like immediately feeling better. Signs it's working: your reaction is becoming different automatically, without having to consciously manage it each time. You stop letting a bad morning mean the whole day is ruined. You lose the intense compulsive urgency to do things. You spend less time thinking and talking about your symptoms, even if they're still present. Flares and setbacks move through you faster. You start doing things regardless of how you feel, without waiting to feel better first.

You will mostly see this looking backward — not in real time. Do not constantly monitor for progress, as that is its own form of hypervigilance.

When to bring tools back

The gauge is desperation. When you no longer feel desperate all the time — when you feel some stability, some capacity, and like you're doing better — that's when you can check whether you could use a tool from self-care vs. from "I need this feeling to stop." If it's the second, it's not time. Desensitize first, then regulate. The regulating tools work beautifully on a desensitized foundation — and they'll actually work, instead of feeling like more pressure.

Q14 • Multiple people asked

Baby steps back into life vs. jumping in — which is the right approach? What does reentering life look like for someone who is severely limited or housebound?

This isn't about baby steps vs. jumping in — it's about the energy behind the doing. Easing in cautiously because you're terrified still signals danger. Doing something small without monitoring it, without overthinking it, without being terrified about how you feel — that's a completely different signal.

For those who are severely limited: you know your capacity. It might be sitting up for a few minutes, looking out a window, walking to the bathroom, baking something, calling someone, sitting outside for five minutes. It doesn't have to be big. It just has to be an energy of living vs. managing.

Ask yourself: what is one small thing I could do today that expands my capacity? Then work on your response to whatever your nervous system does in reaction. The worst thing you can do when severely sensitized is have nothing to engage your attention — because it defaults to your symptoms, and they become your entire world.

Don't wait to feel ready. That day is not coming. Start now while you still feel bad. Your nervous system learns from the doing of new things.

Q15 • Multiple people asked

How do I apply this when I can't remove the stressor? (Ongoing caregiving, difficult family situations, unavoidable chronic life circumstances)

The work is never about removing the stressor. It's about how you respond while the stressor is there. Believing "I can't heal until this situation changes" is itself the resistance — that is the mental fight.

Your nervous system is reacting to your reaction to the stressor. That second layer — your reaction — is what you can change. Some of the most powerful rewiring happens in the middle of ongoing challenging situations, not once they stop.

Important caveat: this does not apply to active abuse situations or situations where you are in actual danger. Know the difference between a genuine stressor and an actual danger. If you're not sure whether your situation truly cannot change, or whether it's just hard to change — that is important work to do, and it's often clearer once you're a bit less sensitized.

Q16 • Multiple people asked

What about dissociation and numbness? How do you allow when you can't feel anything?

Dissociation and numbness are symptoms of a sensitized nervous system. Some systems go super loud (panic, pain); others go quiet (shutdown, numbness, DPDR, dissociation). It's the same alarm — a different expression of it.

Allowing does not require feeling something specific. Allowing numbness IS the allowing — without trying to fix it, solve it, force yourself back into feeling, or see it as categorically different from what others experience.

Trying to force yourself to feel, to get "back into your body" before doing this work, is just another reactive behavior. Look for the ways you're fighting the numbness. Stop fighting it, and your system will

begin to reconnect on its own timeline.

Common Themes Maggie Observed

Theme 1: "Mine is Different"

Almost every question was looking for the exception — mine's 24/7, mine's thoughts, mine's shutdown, mine's a real diagnosis. This is protective: if it doesn't apply to you, you don't have to walk into discomfort. But this IS what sensitization is. The impulse to feel uniquely broken or uniquely unable to heal is one of the most universal features of sensitization. You are not the exception. Burn your list of reasons why. You're not uniquely broken — it just feels that way. And it feels that way for almost everyone.

Theme 2: Questions as Reassurance

Many of these questions are really asking: "Tell me exactly what to do so I know I'm not doing it wrong." That is certainty-seeking — applied now to the course itself instead of to symptoms. The course gives you what you need. Your brain is rejecting how simple it is because it doesn't match the complexity of what you're experiencing. That mismatch is expected. The answer is still the same.

Theme 3: Monitoring Your Response Instead of Your Symptoms

Once you stop watching your symptoms, watch out for transferring that hypervigilance to watching your own response instead. "Am I allowing correctly? Is this progress? Is it getting better?" That's the same pattern with a different target. Transfer your attention from symptoms — not to your healing process — but to your life.

Final Reminder: No One Can Do This For You

There is a big pull to outsource decisions, to want someone to say yes/no/do this/don't do that. So much of this is about you being accountable to yourself, learning from your experience, using the guardrails in the course, and letting this be experiential. Trial and error. Learn from the process.

A second Q&A session is planned for next month. Keep an eye out for the submission form.

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